THE WAITING ROOM STORIES PROJECT

What owners have told us.

235 submissions with permission for anonymous campaign use.
Included responses: 24 May-7 July 2026. Source reviewed: 10 November 2026.

A self-selected collection of experiences, not a representative survey or independently verified clinical evidence. Most included responses are from the United States. These figures are not UK rates.

199 / 235
84.7% said cost was part of the decision (Yes or Partly)

115 / 235
48.9% selected upfront payment as a barrier

107 / 235
45.5% selected a safe payment plan as something that might have helped

Geography: United States 195 (83.0%); United Kingdom 24 (10.2%); Canada 7 (3.0%); other locations or not stated 9 (3.8%).

Barriers and possible help

Each question has a denominator of 235, with no missing answers in this cohort. Multiple answers were possible, so percentages do not add to 100%. Smaller options are grouped as explained on page 2.

Barriers selected

Had to pay upfront	**115** **48.9%**
Surgery or treatment was unaffordable	**42** **17.9%**
Other (selected by respondent)	**29** **12.3%**
Diagnostic imaging was unaffordable	**20** **8.5%**
ICU/hospitalisation was unaffordable	**19** **8.1%**
Insurance did not cover it	**18** **7.7%**
Credit/payment plan was not possible	**16** **6.8%**
Specialist referral was unaffordable	**9** **3.8%**
Insurance limit ran out	**7** **3.0%**
Unclear policy cover or reimbursement wait	**6** **2.6%**
There was no fast emergency fund or grant	**5** **2.1%**

What might have helped

A safe payment plan	**107** **45.5%**
A fast emergency grant	**53** **22.6%**
A charity or fund I could access quickly	**49** **20.9%**
Insurance paying the vet directly	**26** **11.1%**
Other (selected by respondent)	**25** **10.6%**
Insurance that covered more	**21** **8.9%**
Someone explaining the options simply	**14** **6.0%**
More time to decide	**10** **4.3%**
Clearer insurance information before the emergency	**8** **3.4%**

A preference for credit or a grant is not evidence of eligibility, availability or financial suitability.

UK evidence and next action.

24 UK submissions are included. In this small, self-selected subset, 17 selected Yes or Partly when asked whether cost was part of the decision; 6 selected a safe payment plan as helpful and 5 selected insurance paying the vet directly. These support counts can overlap. No UK prevalence estimate is claimed.

A practical proposal

At one willing UK hospital, test a short payment explanation within existing paperwork: the amount needed now, the separate hospital and insurer decisions, what remains uncertain, and who will give the next answer. A possible 20-30-encounter trial would measure owner understanding, unresolved payment barriers and staff time. The provider would retain clinical decisions and patient records. No pilot or partner is agreed.

A clearer explanation cannot by itself make an unaffordable bill affordable. The purpose is to remove avoidable administrative barriers and identify the remaining financial decisions that require change.

How to interpret this release

Source: a snapshot of the connected current-form response Sheet. It contains 295 rows and 294 distinct platform submission IDs; one repeated ID has matching structured answers and permission. The latest timestamp was retained. These are submissions, not verified unique owners, animals or emergencies.

Permission: only 235 records answering Yes, anonymously to the campaign-use question are included. The other 59 record contact-first or private-research-only choices. Follow-up contact permission is separate. The complete historical form notice has not been reconstructed.

Dates: source records extend to 3 September 2026; the included public-use subset ends on 7 July. November is the review date, not a new-response date. Completeness against the live Fillout account is not yet independently reconciled.

Counting: use direct fixed-option answers only, with one count per submission per category. No narrative keyword coding is included. Barriers and support have 29 and 47 multi-option submissions respectively. Percentages are rounded to one decimal.

Small groups: published cells are at least five. Unclear policy cover and reimbursement waiting are combined; each submission is counted once in that group. Smaller location groups are combined. The website explains the additional care, cost and insurance grouping; UK figures are selected counts only.

Correction: the old 402 combined 361 processed rows with 41 legacy entries; the processed rows contained repeated generated IDs. This release uses a different, documented source and permission rule. Legacy entries and Professional Insight are excluded, not discarded.

Limits: self-selection, recall and audience geography limit interpretation. Accounts are self-reported, not checked against clinical records or invoices. These data cannot establish overcharging, national prevalence, comparative provider quality or a financial cause of a medical outcome.

Sources, tables and contact

Full tables, methods and aggregate CSV/JSON: waitingroom.kingchillithepug.com/patterns.html
Independent UK context: CMA veterinary market investigation; RCVS communication and consent guidance.
Project enquiries/corrections: info@kingchillithepug.com. No emergency funding or treatment service.